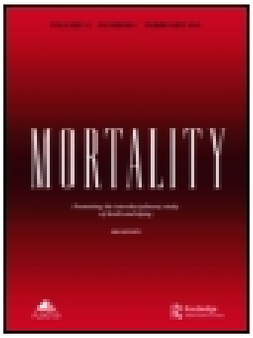

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ISSN: 1357-6275 (Print) 1469-9885 (Online) Journal homepage: http://www.tandfonline.com/loi/cmrt20

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To cite this article: Phillipa J. Malpas & R. Glynn Owens (2016): Given that physician-assisted dying is ethical, should it be part of a doctor's role?, Mortality, DOI: 10.1080/13576275.2015.1137279

To link to this article: http://dx.doi.org/10.1080/13576275.2015.1137279

Published online: 08 Feb 2016.

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Mortality, 2016
http://dx.doi.org/10.1080/13576275.2015.1137279

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Given that physician-assisted dying is ethical, should it be part of a doctor's role?

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ABSTRACT *Much has been written about the ethical issues involved in euthanasia, assisted suicide and other forms of assisted dying. Such discussions have often raised legitimate concerns and challenges to those arguing the case for some form of assisted dying to be legalised. A further concern, though, which relates only tangentially if at all to the ethical issues, is whether or not this is an acceptable part of a doctor's role. The present paper outlines some of the concerns that have been raised regarding such a change or extension of role, and considers them from historical, societal, empirical and professional perspectives. It is concluded that there is no strong argument, in principle, which precludes assistance in dying from being part of a doctor's role.*

KEYWORDS: assisted dying; doctor; euthanasia; death; dying

Introduction

The prohibition on health professionals assisting their patients to die stands as a central tenet of most professional medical associations worldwide and underpins many codes of practice and ethics. The ethical and legal underpinnings for such a prohibition rest primarily on the principles of non-maleficence and beneficence. *Prima facie*, to hasten the death of one's patient irrevocably harms the interest a person has in continued life and offends against the sanctity of life. Yet one's interest in continued life may be trumped by the greater interest one has in not suffering – something that may only be relieved by death.

In certain qualified circumstances a physician assisting a patient to die is, in principle and practice, accepted in many countries and jurisdictions around the world as ethically and legally permissible. The term physician-assisted suicide (Materstvedt et al., 2003) refers to the practice of a doctor assisting a patient to die by giving them the means to end their own life at the patient's explicit request, and is legally and ethically permitted in five states of the United States: Oregon (1997), Washington (2008), Montana (2009), Vermont (2013) and New Mexico (2014). However, the term (physician assisted suicide) is contentious[1]: (Quill, 2012) it may be seen to imply that an individual has lost

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the ability to make rational decisions, and it has associations with mental ill-health.

The practice of assisting a patient to die by actively and intentionally ending their life at their explicit request is permitted in some European countries: The Netherlands (2002), Belgium (2002) and Luxembourg (2008). It is generally referred to as euthanasia (Materstvedt et al., 2003). All three countries also permit physician-assisted suicide. The Canadian province of Quebec recently allowed a law to permit doctors – in qualified circumstances – to administer and prescribe a lethal dose of drugs to competent patients who requested such an action (Hivon & Barrette, 2014), and in February 2015, the Supreme Court of Canada ruled that prohibiting assisted suicide is unconstitutional (Woolf, 2015). Federal and provincial governments have one year to enable legislation that will respect the Supreme Court ruling. Switzerland (1942) permits assisted suicide which may or may not be physician-assisted, but prohibits euthanasia (Griffiths, Weyers, & Adams, 2008). It is illegal in every jurisdiction in the USA for a doctor directly to assist a patient to die by administering a lethal dose of medication to them.

Doyal rejects the commonly made moral distinction between active (euthanasia) and passive (physician-assisted suicide) acts that hasten death arguing that there is no relevant difference between them (Doyal & Doyal, 2001). Dworkin and Frey (Dworkin, Frey, & Bok, 1998) claim that in cases of physician-assisted suicide and euthanasia, the doctor and patient act together; the two acts only differ in who acts last. Quill concurs that in the case of physician-assisted suicide, whilst the patient must be able to take the lethal medication by their own hand, 'the prescribing physician is morally and legally responsible as an accomplice' (Quill, 2012). For the purposes of this discussion, the practices of physician-assisted suicide and euthanasia will be referred to as physician-assisted dying (PAD) as the intention of the patient and the doctor is the patient's death.

Intense political and social debate about PAD is currently being played out in a number of countries around the world, including New Zealand, the UK, France, Canada, the USA and Australia; however most professional medical organisations in these countries decline to engage with the topic (Bosshard et al., 2008) and some are clearly opposed to such practices. The American Medical Association states that PAD is 'fundamentally incompatible with the physician's role as healer and would pose serious societal risks'.[2] The New Zealand Medical Association[3] states that PAD is unethical in both concept and practice, explicitly stating that even if the law would recognise the legal permissibility of PAD, the association would 'continue to regard them as unethical'.

In a recent British Medical Journal editorial, three editors supported Lord Falconer's Assisted Dying Bill that would allow adults who are expected to live less than six months, to be provided with medication to end their lives (Delamothe, Snow, & Godlee, 2014). The British Medical Association remains firmly opposed to legalising PAD.

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In the following discussion we briefly defend the position that the ethical acceptability of PAD as a last resort option at the end of life is, in principle, settled. We then critically consider the objection that whilst this may be established in principle, PAD is not part of the role of the doctor (Kass, 1989; Gaylin, Kass, Pellegrino, & Siegler, 1988; Breitbart, 2012) nor the business of medicine (Randall & Downie, 2010). Whilst we acknowledge the role of other health professionals within the arena of PAD (Kuhse & Singer, 1993), for the purposes of this discussion, we focus our attention on the role of doctors.

Ethical Permissibility of PAD

In many countries it is recognised that an adult of sound mind has a legal and moral right to refuse any medical treatment and to withdraw from existing medical treatment, even if their decision goes against medical recommendations, and will hasten their death. Thus for instance, a person may request to be removed from the dialysis schedule when they determine that the benefits from dialysis are outweighed by the burdens which such treatments impose on their quality of life. Philosophically, the principle of respect for individual autonomy is paramount – a competent adult should not have to endure medical treatment that he or she believes can no longer benefit him or her (Warnock & Macdonald, 2009). Individuals have a right to be left alone and 'to make decisions about his or her own body on any level' (Morgan & Sutherland, 1998). The recent death of Brittany Maynard in Oregon highlights an individual's right to decide when their quality of life is no longer acceptable to them (Anonymous, 2014; Angell, 2014). Brittany exercised that right under Oregon's Death with Dignity Act which allows physician aid-in-dying.

The creation of an advance care plan with one's physician or the enactment of instructions in an advance directive provides individuals with some certainty about their medical preferences around medical treatment and care at the end of life and gives further evidence of autonomy's reach. At times a person's medical decision-making per their instructions, may hasten their dying (for instance, the decision not to be resuscitated or to decline certain medications). Such instructions are generally held to be consistent with both law and ethics and reflect growing recognition that a humane death is not as simple as managing pain relief adequately. It entails looking to what the patient says makes their life meaningful and worth living (Boisvert, 2010).

The significance and centrality of respecting the autonomous decisions of individuals near the end of life, underpins the argument for the ethical permissibility of PAD as a last resort option at the end of life. The argument has been strongly advanced that there is no valid ethical distinction between the actions of a doctor who complies with a competent patient's request to be removed from ventilator support (and the patient dies), and the same doctor who administers a competent terminally ill patient an assisted death (on request) via a lethal injection (Rachels, 1974; Brock, 2001).

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Assuming for the sake of argument that this view is reasonably settled, it still leaves unresolved whether such actions are part of a doctor's role, or an extension of it. It is in this direction that we now turn.

The role of the doctor

Although it might be argued that involvement in assisted dying runs contrary to the fundamental role of the doctor, (that of healing and preserving life), such an argument becomes difficult to sustain when closely examined. A simple approach to arguing this would be to assert that the deliberate ending of a patient's life is in breach of the medical practitioner's 'Hippocratic Oath'. In answering such an objection one might most obviously call into question the relevance to the present day of a policy thought to date back some twenty-five centuries. What was appropriate in the times of the Ancient Greeks cannot simply be assumed to be similarly appropriate in the twenty-first Century. Indeed, a moment's reflection will illustrate that in any case the injunctions of the oath have already been overridden with respect to practices such as surgery and abortion – and it is unlikely that many medical practitioners regard, as the Oath directs, the children of their teachers as brothers, nor is it common practice to provide instruction in medicine free of charge to these 'brothers'. Appealing as it may be to cede the profession's autonomy to one of its early practitioners, to do so would be no more logical than to expect modern chemists to have their practice guided by that of the alchemists.

Recent discussions of the role of the doctor have recognised both the changes in the problems for which doctors provide treatment (baldness, sexual dysfunction, antisocial behaviours (Caddell & Newton, 1995)), and in the societal context within which their services are delivered. Thus we find in the modern literature assertions such as:

> The proper role for the physician is to provide care' that is, to preserve the human essence of their patients and assert the unique human possibility of experiencing meaning even in the last moments of life, while providing amelioration of suffering, and respecting the autonomy of patients to refuse or stop life prolonging measures. (Breitbart, 2012)

> The medical ethos entails respecting the patient's autonomy and well intentioned decision-making, accompanying him or her in situations in which healing has become impossible, and taking into account his or her personal values and priorities. (Martin, Mauron, & Hurst, 2011)

Neither quote necessarily defends PAD per se; both however articulate powerfully the contemporary role of the doctor and the significance of respecting patient autonomy. We argue that PAD has a place in medical care at the end of life when a patient's personal values and priorities are taken into account. Thus such practices are entirely consistent with the doctor's role.

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Within the context of discussion on PAD, other commentators argue that assisted dying has no legitimate place in a doctor's practice (xxx, 2014; Marcoux, 2010; Kass, 1992). Randall and Downie claim the involvement of doctors as decision-makers in the practices of euthanasia and assisted suicide flatly contradict what it means to be a doctor (Randall & Downie, 2010). Others take a more neutral stance, leaving the decision to proceed (or not) with a request for an assisted death with the doctor. Under this view, such involvement becomes a conscientious decision, concerned with one's own moral or religious code as it is with other factors such as the depth of one's relationship with the patient (much like the provision of abortion and sterilisation services).

Despite this, there is clearly a significant minority within the profession who do not see assisted dying as fundamentally incompatible with their role, given that they are willing to perform such acts even when illegal (Mitchell & Owens, 2004; Kuhse, Singer, Baume, Clark, & Rickard, 1997; Van Der Maas, Van Delden, Pijnenborg, & Looman, 1991; Taylor, 2015; Malpas, Mitchell, & Koschwanez, 2015).

It appears then, that the current debate involves a range of assertions regarding the 'proper' role of the doctor, together with observations of actual practice; these two do not always remain entirely concordant. Before asking what is the appropriate role of the doctor it might justifiably be asked 'who is do decide?' Clearly, in any individual case then, the individual has a reasonable entitlement to refuse to take part in any act which would be contrary to their own deeply held principles. Similarly it is reasonable to expect that practitioners would have some rights to restrict the activities of their colleagues when they had good evidence or reason to expect that these activities would impact on themselves (e.g. by affecting their own relationship to their patients). Unlike the first example, though, where a case could be argued simply for 'opting-out' without any need for greater justification than '… it's against my principles …', it is arguably only fair that an attempt to impose those principles on others should require a reasonably rigorous standard of evidence in support of the case being made. Thus, in a situation where some practitioners are willing to provide assistance in dying and others are not, then it is likely that this will, to some extent, influence patients' choice of physician. But arguably it is better for everyone concerned if physicians and patients hold similar views to each other; a patient who does not believe in contraception or abortion may feel more comfortable with a doctor whose views are concordant with their own, and vice versa. Certainly there is no reason to assume that any existing good relationship between a doctor who does not believe in assisted dying and a patient who shares those beliefs will be affected by knowing that on the other side of town there is another doctor who provides assistance in dying to patients who support such practices. At the very best, the argument that 'the profession providing euthanasia will destroy the doctor–patient relationship' can only be used to justify individuals refusing to become involved in assisted dying where they feel it will affect their own relationships. It cannot be used to justify preventing others from providing such services (Buchalter, 2009).

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Impact on other professional areas

Perhaps a more legitimate concern regarding the provision of assisted dying is that the legalisation of euthanasia could lead to a diminution of other services provided at the end of life – in particular that it might result in lack of support for palliative care, or adversely affect the quality of palliative care services. Such arguments might be seen to have an intuitive appeal – surely, it could be argued, governments will opt for the cheaper alternative of assisted dying rather than investing money in/on palliative care services? This argument, however, involves an assumption that palliative care and assisted dying are alternatives, and that more of one will somehow involve less of the other. Yet both logic and experience suggest that this assumption is unwarranted. In countries where legislation permits any form of assisted dying, there is a consistent requirement that before assistance can be given, all other options must be considered. Clearly if palliative care is a reasonable and effective alternative to assisted dying in a particular case, this can only be considered if resources are available. Logically, if it is considered that palliative care might be an alternative course of action, assistance in dying could not legally be provided until this had been offered.

In practice, the evidence is clear that the palliative care/assisted dying pairing is not in any sense a dichotomy, and there is evidence that both can coexist effectively (Bernheim et al., 2008; Bernheim, Distelmans, Mullie, & Ashby, 2014b), both in countries where assisted dying is legal and those where it is not (Saunders, 2008). In New Zealand, for example, the overwhelming majority of physicians who admitted (illegally) helping their patients to die had access to palliative care services (Mitchell & Owens, 2004; Malpas et al., 2015); clearly the availability of such services is not seen as meeting the needs of every terminally ill patient. Nor does the provision of legally accepted assistance in dying impair the development of palliative care; in the Netherlands and Belgium, for example, there has been a substantial increase in the availability of palliative care services in the years during which euthanasia has been available (Bernheim et al., 2014b; Bernheim, Chambaere, Theuns, & Deliens, 2014a). Evidence from both Oregon and Washington states show that the majority of individuals who used the Death With Dignity Act to end their lives were enrolled in hospice or palliative care (Washington State Department of Health, 2013; Oregon Government, 2014). Moreover, professional 'high care' hospices in the Netherlands, whilst not providing euthanasia themselves, will refer patients who insist to an institution which does. Thus, it is possible to provide specialist palliative care whilst at the same time accepting a patient's right to assistance in dying. Clearly, the assumption that palliative care and assisted dying are opposed is questionable in terms of practical experience.

A further concern that society's trust in the medical profession and the doctor–patient relationship would be undermined were PAD to be legally permitted (Saunders, 2008) needs further elucidation. The doctor's obligations of trust and seeking the patient's good are a central component of the role and

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underpin a defining aspect of the medical profession (Australian Medical Association, 2011; New Zealand Medical Association, 2011). This is especially so at the end of life where individuals and their families may be particularly vulnerable.

Sobel (Sobel, 1996) claims that the role of the doctor is to advocate for the patient, 'fighting against disease regardless of transitory pain, depression or fear'. According to Sobel, that sacred trust is predicated on the conviction that the doctor is a 'stalwart ally' of the patient. To permit PAD, it is argued, is to thwart the advocacy role and abandon the fight, resulting in a loss of trust or confidence. Others concur, agreeing that patients will struggle to believe that doctors have their best interests at heart when they (doctors) are permitted to end the lives of their patients (Gaylin et al., 1988; Pellegrino, 1992). However this vision of medicine is inherently paternalistic, lying within a tradition of medicine that posits that the doctor knows best and will do everything to sustain life. Yet far from undermining trust, the doctor who 'listens carefully to the expressed desires of a patient and then honestly responds in conformity to those desires would most likely be a physician that gains, rather than loses patient trust' (Momeyer, 1995; Sumner, 2011).

Furthermore studies conclude that the public (Lindblad, Lofmark, & Lynoe, 2009; Hall, Trachtenberg, & Dugan, 2005; Graber, Barceyi, Robertf, & Roberta, 1996), and patients (Graber et al., 1996), do not believe PAD will erode trust in medical services; for instance one study found that trust in medical services would increase if PAD were legally available at the end of life (Lindblad et al., 2009).

Conclusion

In principle and in practice, physician-assisted dying is legally available and ethically sanctioned in a number of jurisdictions around the world. Here, the doctor is pivotal in both the practice of euthanasia and assisted suicide, and such practices are viewed as part of the role in much the same way that practices such as abortion and sterilisation are regarded. Furthermore in countries where such practices are illegal, evidence shows that some physicians assist their patients to die, confirming the view that physicians themselves consider such assistance as part of their role in caring for patients at the very end of life.

Over the past decade many jurisdictions have allowed PAD in qualified circumstances, recognising that for a small number of patients, a medically assisted death aligns with the role and responsibilities doctors have towards their patients at the end of life.

Of course, a case can be made that certain actions may be inappropriate as part of a doctor's role even if legally authorised and actually practised by some practitioners in different countries or times. Perhaps the most obvious of these is the involvement of doctors in the death penalty – an action which may be legal and indulged in by some, but which nonetheless could be (and by many medical organisations is) seen as an entirely inappropriate role. There is here,

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however, a critical difference, in that the person being executed is not in any meaningful sense a patient of the physician concerned. Etymologically the term 'patient' itself indicates a person who is suffering, and the practice of medicine is arguably one directed towards the relief of such suffering. In the case of capital punishment, a patient is not being treated, and whilst medical skills are being used, what is being practised is not medicine. In the case of assisted dying, the person is indeed a patient, is indeed suffering, and the application of medical skills in the relief of such suffering can be seen as entirely consistent with the doctor's role *qua* doctor.

Seen together with societal changes that prioritise the autonomy and interest of individuals, and recognising that many of the concerns predicted in the wake of PAD being legalised have not, in those countries which have legalised it, been realised, we conclude there is no strong argument, in principle, which precludes assistance in dying from being part of a doctor's role.

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Disclosure statement

Phillipa J. Malpas declares that she is a member of the End-of- Life Choice Voluntary Euthanasia Society of NZ. Both authors declare no financial conflicts of interest.

Funding

Phillipa J. Malpas's research has been generously supported by a grant from the Health Research Council of New Zealand (Grant number 12/657). The funding source had no involvement in the study design, research, and in the writing of the article or in the decision to submit it for publication

Notes

[1] Those who object to its use claim that the focus should not be on the physician who provides the lethal means, rather it should remain on the patient who makes an informed decision both to request a lethal dose of medication and take it at a time of his/her choosing. Moreover in the jurisdictions noted above, the doctor is not legally required to be with the patient when they self-administer the lethal medication, further distancing the doctor from the patient's death. Finally, the term is not used in Oregon, Washington or Vermont Death with Dignity laws.

[2] http://www.ama-assn.org//ama/pub/physician-resources/medical-ethics/code-medical-ethics/opinion2211.page.

[3] http://www.nzma.org.nz/policies/advocacy/position-statements/euthanasia.

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Biographical Notes

Phillipa Malpas works as a clinical medical ethicist at the Auckland School of Medicine, University of Auckland. She is interested in the ethical dimension of death and dying, particularly around individuals' attitudes towards medical assistance to hasten death, and the way in which these may influence patient's expectations of medical treatment and care at the end of life.

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